



Table of Contents

Page Title

Did You Know?

1 Licensure Requirements

All Providers That Use a Trading Partner

2 EDI Vendor Transition

All Providers That Utilize the ColoradoPAR Program

3 What is the ColoradoPAR Program?
3 Acentra Provider Satisfaction Survey
3 Interoperability Changes January 2026
4 Pediatric Long-Term Home Health Services
4 PAR Submission Training for Acentra
5 Pediatric Behavioral Therapy Form Change

Behavioral Health

5 CCBHC Updates
6 Colorado Intensive Home-Based Treatment
Potential Provider Forums
7 H0011 Fee Schedule Updates
7 Reminder: State-Directed Payments

Dialysis

8 Mircer® (Epoetin Beta)

Durable Medical Equipment

8 Mechanical Stretching Device Criteria
10 OT and PT: Ordering Equipment Covered
by Third-Party Payer

Home and Community-Based Services (HCBS), Home Health

10 Consolidated HCBS Specialties
11 Prohibition of Concurrent Delivery of HCBS
Developmental Disabilities Waiver
12 2026 Base Wage and Workforce Reporting

Hospital

12 Hospital Stakeholder Engagement Meeting
13 RHC Stakeholder Engagement Meeting

Immunization

13 Updates and Reminders

Managed Care

14 New Escalation Request Form

Outpatient Therapy

14 POC Signature Requirement Revisions

Pharmacy and All Medication Prescribers

14 LexisNexis Pharmacy Prescriber Validation

Physician Services

15 Free SBIRT Training
16 Lactation Support Services

Primary Care Medical Providers

17 Access Stabilization Payment Update

Transportation

17 TransDev Provider Reminders
18 MediDrive Enrollment for TransDev Only
18 Non-Emergent Medical Transportation
(NEMT) Upcoming Broker Communications
19 NEMT Broker Transition Webinar
20 Reminder to All NEMT Providers

Provider Training Sessions

21 May 2026 Schedule

Did You Know?

Providers **must** verify the status of their license on file with Health First Colorado (Colorado's Medicaid program). Effective May 18, 2026, claims for providers with expired licenses will deny for Explanation of Benefits (EOB) 3385 - "Provider license not active on date of service."

Not all licenses can be updated automatically in the system. Providers can update the license in the [Provider Web Portal](#) by doing a Provider Maintenance Request. Refer to the [Update Licenses and Clinical Laboratory Improvement Amendments \(CLIA\) Quick Guide](#) for instructions on adding or updating a license.

Providers must resubmit any previously denied claims once license updates are completed.

All Providers That Use a Trading Partner

Electronic Data Integration (EDI) Vendor Transition

Providers that use a clearinghouse or billing agent to submit X12 batch transactions on their behalf will need to ensure that the vendor is aware of the changes below.

Electronic Data Integration (EDI) functionality, including batch processing and trading partner enrollment, is transitioning to a new platform and vendor; Edifecs, a Cotiviti Business.

These changes are being implemented in three (3) phases.

The initial phase, [X12 File Naming Standards changes](#), went live on January 7, 2026.

The next phase is targeted for implementation in the summer of 2026 and includes the following changes:

- New agreement to be signed by trading partners
- New platform for trading partners to exchange files
- New login credentials for the platform (to be issued once the agreement has been acknowledged)
- New Trading Partner Enrollment and Testing Site



Trading partners will be invited by email to participate in one (1) of multiple training groups. Group training sessions will start later this spring and will continue until implementation. Ensure [contact information is current](#) to receive the training invitation.

The final phase is expected to be implemented in the fall of 2026.

What is staying the same?

- Providers may still use the [Provider Web Portal](#) to submit individual claims, maintain enrollment, verify individual eligibility and more.
- Providers and trading partners will contact the [Provider Services Call Center](#) with any questions about claims, the Provider Web Portal or EDI.
- Active trading partners will retain the same trading partner IDs.

Note: Trading partners will **not** be able to utilize the Provider Web Portal going forward. Trading partners currently using the Provider Web Portal should switch immediately to the Move-It FTP server for X12 file exchanges. Providers may not assign a trading partner role in the Provider Web Portal.

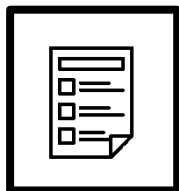
Refer to the [Electronic Data Interchange \(EDI\) Support web page](#) and [Colorado Medicaid Enterprise Solutions \(CMES\) Transition web page](#) for more information.

All Providers That Utilize the ColoradoPAR Program

What is the ColoradoPAR Program?

The ColoradoPAR Program is a third-party, fee-for-service Utilization Management (UM) program administered by Acentra Health, Inc. Visit the [Colorado Prior Authorization Request Program \(ColoradoPAR\) web page](#) for more information about the ColoradoPAR Program.

Acentra Annual Provider Satisfaction Survey



Acentra Health and the Department are announcing the opening of the Colorado Prior Authorization Request [\(PAR\) Provider Survey](#) for all providers that work with Acentra Health and use the Atrezzo® provider portal. The Colorado [PAR Provider Survey](#) opens April 13, 2026, and will remain open through May 22, 2026.

The Colorado PAR Provider Survey is an opportunity to provide feedback regarding Acentra Health services in processing PARs, customer service, provider education and timeliness.

Interoperability Changes Began January 2026

Significant changes in Pend and Prior Authorization Request (PAR) timeframes:

Several requirements set forth in the Centers for Medicare & Medicaid Services (CMS) Interoperability Final Rule have been implemented as required. The most notable changes impacting providers are highlighted here:

- Pends for additional information are **reduced** from 10 business days to seven (7) calendar days.
- Additional pends on the same PAR will not be extended, all information requested in the initial pend must be supplied, or the PAR will result in a technical denial.
- Expedited PARs: **pends or requests for information** will not be allowed in order to comply with CMS three (3) calendar day Turn Around Time (TAT) requirement. Please ensure all documentation is included with the initial PAR submission.

These changes are required for Acentra Health, Inc. and the Department to be compliant with the Interoperability Final Rule. This will facilitate a faster standard turnaround time for PARs, reducing it to seven (7) calendar days. All PARs, including pends, should be processed within 21 calendar days of initial submission. Visit the [CMS Interoperability and Prior Authorization Final Rule web page](#) for more information on the rule.

Pediatric Long-Term Home Health (LTHH) Services

Final Deadline and Enforcement of Prior Authorization Requests (PARs) for Pediatric Long-Term Home Health (LTHH) Services

The final requirements for Prior Authorization Requests (PARs) for Pediatric Long-Term Home Health (LTHH) services are being implemented. Effective May 4, 2026, PARs are required for all new and existing members receiving Pediatric LTHH services, including Physical Therapy (PT), Occupational Therapy (OT), Speech Therapy (ST), Skilled Nursing (RN/LPN) and Certified Nursing Assistant (CNA) services. Providers must submit PARs through the Atrezzo® Provider Portal in accordance with previously-issued guidance.

Beginning June 1, 2026, PAR compliance will be fully enforced. Services rendered without an approved PAR may be subject to claim denial or nonpayment, payment suspension and additional compliance actions, which may also result in disruptions to member services.



Providers are expected to:

- Submit PARs timely and in alignment with established requirements.
- Respond promptly to requests for additional information or medical necessity determinations.

The Department will continue to support providers through training and technical assistance in partnership with Acentra Health, as well as ongoing updates via the LTHH and ColoradoPAR web pages.

Contact homehealth@state.co.us with questions regarding LTHH benefits. Contact COProviderIssue@acentra.com with PAR submission assistance.

Prior Authorization Request (PAR) Submission Training for Acentra

Acentra Health will provide benefit-specific Prior Authorization Request (PAR) submission training for all providers and benefit-specific training for Physician Administered Drugs (PAD). The training dates and times are listed below in Mountain Time:

- [PAD Provider Benefit Training May 13th, 2026, at 9:00 a.m.](#)
- [PAD Provider Benefit Training May 13th, 2026, at 12:00 p.m.](#)
- [Portal Registration and PAR Submission Training May 27, 2026, at 9:00 a.m.](#)
- [Portal Registration and PAR Submission Training May 27, 2026, at 12:00 p.m.](#)

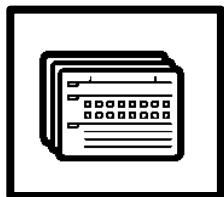
PAR submission training sessions are appropriate for all new users and include information on how to submit a PAR using Acentra's provider portal, Atrezzo®.

Contact COProviderIssue@acentra.com with questions or if needing assistance when registering for Atrezzo training or accessing the portal. Visit the [ColoradoPAR Training web page](#) for additional training information.

Providers of Pediatric Behavioral Therapy (PBT)

Change of Provider Forms and 97151

Instances have been identified in which multiple Pediatric Behavioral Therapy (PBT) providers have submitted claims and obtained PAR approvals for CPT code 97151 for the same member within a single 365-day period. Current policy allows multiple providers to bill CPT 97151 in support of member choice; however, this practice results in duplicative or overlapping Prior Authorization Requests (PARs) and claims.



Beginning June 1, 2026, if a PBT provider submits a PAR request with code 97151 for a member who already has an active PAR with another provider, the requesting provider must submit a completed Change of Provider form to ensure accurate billing and claims processing. The form can be found on the [Provider Forms web page](#) under the PAR Forms section. If a form is not submitted or not completed, the PAR will be pended back for additional information to the requesting provider.

Contact HCPF_EPSDT@state.co.us with questions regarding this change.

Behavioral Health Providers

Colorado Certified Community Behavioral Health Clinic (CCBHC) Updates

2026 Certified Community Behavioral Health Clinic Demonstration Application Submitted

On March 31, 2026, the State of Colorado submitted an application for the federal Section 223 Certified Community Behavioral Health Clinic (CCBHC) Demonstration. This demonstration supports expanded access to comprehensive, community-based behavioral health services for Health First Colorado members.

The [Notice of Funding Opportunity](#) (NOFO) was released on March 6, 2026. Colorado solicited public comment on the draft application from March 20 to March 24, 2026, and submitted the final application on March 31, 2026. The Department appreciates those who took the time to review and provide input on the application.

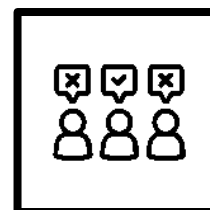
States selected to participate in the 2026 Section 223 CCBHC Demonstration are expected to be announced in June 2026. Updates will be posted on the Department [Certified Community Behavioral Health Clinics \(CCBHC\) web page](#).

Prospective Payment System (PPS) Accountability Guidelines Stakeholder Engagement

The Department, in partnership with the Behavioral Health Administration (BHA) and community providers, continues to refine policies and procedures to support Colorado's potential participation in the Section 223 CCBHC Demonstration.

A [PPS is a flexible, advanced reimbursement model](#) that ties payment to daily encounters instead of to individual services. It is provider-specific and based on each provider's unique cost structure, designed to cover a safety net provider's actual cost of services through a cost-based, encounter-driven model. A report assessing Colorado's PPS design is available on the [CCBHC Planning Grant web page](#). The draft [CCBHC PPS Accountability Guidelines](#) is available for ongoing public comment through the [CCBHC Feedback Survey](#) on the CCBHC Planning Grant web page.

The first PPS Accountability Guidelines (originally called PPS Guardrails) public stakeholder session took place in January 2026. The next stakeholder session will be held May 8, 2026, at 10:30 a.m. MT (register via [Zoom](#)), with additional updates planned for late May 2026.



Updates will be posted on the [CCBHC web page](#). Submit any feedback on CCBHC via the [CCBHC Feedback Survey](#) or HCPF_BHBenefits@state.co.us, and visit the web page for updates.

The May 25, 2026, CCBHC Steering Committee meeting is canceled in observance of Memorial Day. Visit the [CCBHC Planning Grant web page](#) for information on how to participate in additional upcoming stakeholder engagement opportunities.

Colorado Intensive Home-Based Treatment Potential Provider Forums

Colorado System of Care (CO-SOC) is looking to expand and grow the provider network. This initiative focuses on strengthening the workforce and ensuring model fidelity to enhance services to the most vulnerable children and youth, empowering them to lead healthier, more fulfilling lives. These forums are open to anyone, specifically targeting agencies or providers who are wanting to join the Colorado System of Care. No registration is needed.

Colorado Intensive Home-Based Treatment (CO-IHBT) Potential Provider Forums

A national expert, Dr. Rick Shepler, will be presenting on Colorado Intensive Home-Based Treatment (CO-IHBT), a framework and set of practice standards for delivering intensive mental health services to youth with serious emotional disabilities and their families. CO-IHBT reduces the need for more restrictive out-of-home treatment, with a focus on mental health stabilization, safety promotion and support for the family system.

The Department will be presenting on the CO-SOC philosophy, the partnership with the Regional Accountability Entities (RAEs) and the opportunities for funds to support expansion through the Workforce Capacity Center (WCC) at Colorado State University. Dr. Shepler will be presenting on CO-IHBT model components and the intersection of CO-IHBT and Intensive Care Coordination using Enhanced High Fidelity Wraparound.

Forum dates and times are listed below. No registration is needed.

- Thursday, May 7, 2026, 2:00 p.m. to 3:30 p.m. Mountain Time (US and Canada) [Join the May 7 Zoom meeting](#)
- Wednesday, June 17, 2026, 1:30 p.m. to 3:00 p.m. Mountain Time (US and Canada) [Join the June 17 meeting](#)

Contact hcpf_co_soc@state.co.us with any questions. [Subscribe](#) to the Colorado System of Care (CO-SOC) for Children and Youth Behavioral Health Newsletter to receive ongoing updates.

H0011 Fee Schedule Updates

The rates for procedure code H0011 on the Fee-For-Service Behavioral Health Fee Schedules for July 1, 2024, July 1, 2025, October 1, 2025, and April 1, 2025, were misaligned with the correct rates within the Medicaid Management Information System (MMIS). No claim adjustments will be necessary, due to the MMIS having the correct rates. Refer to the [Provider Rates and Fee Schedules web page](#) for all updated Fee-For-Service Behavioral Health Fee Schedules.

Refer to the [Fee-for-Service Behavioral Health Benefit Billing Manual](#) for more information on billing Fee-For-Service Behavioral Health Services. Refer to the [State Behavioral Health Services Billing Manual](#) for more information on the requirements of billing procedure codes for behavioral health services. Visit the [State Behavioral Health Services Billing Manual web page](#) for more information on the requirements of billing procedure codes for behavioral health services.

Reminder: Discontinuation of State-Directed Payments for Behavioral Health Essential Safety Net Services in a Bedded Facility



Effective July 1, 2026, the Department of Health Care Policy and Financing (the Department) will no longer set directed payments for behavioral health essential safety net services in a bedded facility. Directed payments for Mobile Crisis Response services and Opioid Treatment Programs (OTPs), as well as targeted population directed payments, will remain in place until further notice.

When launching the safety net in 2024, directed payments were utilized as a time-limited method to infuse funds into the behavioral health safety net system to support providers through the transition to the new system, support network access and create growth for specific services. Directed payments are reviewed on a regular basis to monitor their effectiveness and are not a sustainable funding mechanism within a managed care model.

The factors that inform when a Directed Payment is considered include:

- When a service is new or is being underutilized across the state

- When there are identified access issues related to a specific service

Regional Accountable Entities (RAEs) may negotiate provider rates based on quality and network needs as a directed payment is phased out.

This policy update will be reflected in Appendix D of the July 2026 edition of the [State Behavioral Health Services Billing Manual](#).

Email hcpf_bhbenefits@state.co.us with any questions.

Dialysis

Mircera® (Epoetin Beta)

Effective May 1, 2026, Mircera® will be considered a Routine Drug for Dialysis. It is also known as Epoetin Beta and is reported using codes J0887 and J0888.

Refer to the [Dialysis Billing Manual](#) for more information regarding Routine and Non-Routine Dialysis.

Durable Medical Equipment

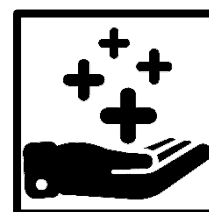
Mechanical Stretching Device Criteria

Effective June 1, 2026, requests for mechanical stretching devices billed under Healthcare Common Procedural Coding System (HCPCS) E0808, E1800-E1808, E1810-E1816, E1818, E1822-E1823, E1825-E1832, L3900 and L3901 will be reviewed using updated coverage criteria.

These criteria can also be found in the [Durable Medical Equipment, Prosthetics, Orthotics and Supplies \(DMEPOS\) Billing Manual](#).

Overview

Mechanical stretching devices, or extension and flexion devices, are used for the treatment of joint contractures in lower and upper extremities. These devices are intended to maintain or restore range of motion and replace some therapy-directed sessions through consistent and frequent joint mobilization in an outpatient setting or in the home.



Eligibility

For members with acute/subacute neurological injuries or orthopedic/post-operative care of less than four (4) months:

- Mechanical stretching devices are covered for eligible members whose medical necessity for this intervention are supported by the submitted clinic documentation and who:

- Suffer from joint stiffness and are not responding to, or as a complement to, physical or occupational therapy during the subacute injury or postoperative period; or
- Have a documented history of motion stiffness or loss in a joint, have had surgery or procedure done to improve motion to that joint, and are in the acute postoperative period following a second or subsequent surgery or procedure.

Members with neurological injuries or have orthopedic/post-operative status equal to or greater than four (4) months may be approved for a two (2)-month trial based on individualized physician review.

Following the two (2)-month trial for patients requesting continuation of coverage, the documentation from the requesting physician or therapist must show:

- Compliance with use (a minimum of 30 minutes of use per day) for a minimum of 70% of days of the trial period, and
- Change in the range of motion measured in the same units as submitted with the initial request (for direct comparison).

Physician review for ongoing approval will focus on whether documentation supports a reasonable expectation that ongoing device use will lead to a functionally meaningful increase in range of motion to facilitate improved skin hygiene or improved ability to complete activities of daily living.

Required Documentation

The following documentation must be submitted for each request when evaluating a member for dynamic splinting systems, static progressive stretch (SPS) devices or patient-actuated serial stretch (PASS) device use:

- A signed order
- A face-to-face visit with a physician addressing the specific joint concern within the prior four (4)-six (6) months
- An occupational or physical therapy evaluation from within the prior four (4) months
- Either the therapy evaluation or the physician note must include all of the following:
 - A modified Ashworth scale to reflect the degree of spasticity or increased muscle tone
 - The angle(s) of the contracture(s)
 - The clinical history of the member's joint stiffness and prior treatments, including physical or occupational therapy, any surgeries and other procedures
 - The duration for which the member has had spasticity and/or contracture(s)
 - The location of each contracture including specification of side and joint

- What medications are being used or have been trialed to address spasticity
- What medications are being used or have been trialed to address pain
- Documentation of a minimum 15-minute trial of the device to support the ability to tolerate the intervention.

Contact Alaina Kelley at Alaina.Kelley@state.co.us with questions.

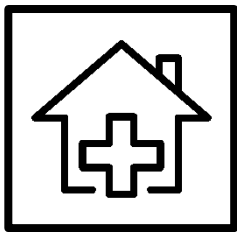
Occupational Therapist (OT) and Physical Therapist (PT): Ordering Equipment Covered by Third-Party Payer

Durable Medical Equipment (DME) and Complex Technology Rehabilitation (CRT) providers are reminded to ensure that all providers listed on a claim are enrolled with either the Third-Party Liability (commercial) insurance provider or Medicare, as well as Health First Colorado, if either Medicare or commercial insurance covers the requested equipment.

Contact Alaina Kelley at Alaina.Kelley@state.co.us with questions.

Home and Community-Based Services (HCBS), Home Health Providers

Consolidated Home and Community-Based Services (HCBS) Specialties



Effective April 29, 2026, eight (8) Home and Community-Based Services (HCBS) specialties were condensed into four (4) specialties. This change allows existing HCBS providers to expand the populations they serve and creates a more efficient enrollment process for new providers.

The specialties that will be condensed have the same service type and similar provider enrollment requirements. The updated four (4) specialties and associated waivers are:

- **Specialty 607: Assistive Technology - Persons with Brain Injury (BI)/Children's Extensive Support (CES)/Supported Living Services (SLS)**
 - Specialty 603: Assistive Technology - BI will be condensed into Specialty 607.
- **Specialty 643: Electronic Monitoring - BI/Community First Choice (CFC)/Complementary and Integrative Health Waiver (CIH)/Community Mental Health Supports (CMHS)/Elderly, Blind & Disabled (EBD)/SLS**
 - Specialty 668: Personal Emergency Response - SLS will be condensed into Specialty 643.
- **Specialty 651: Home Modify-Adaptation - BI/CES/CIH/CMHS/EBD/SLS**
 - Specialty 648: Home Accessibility Adaptations - CES/SLS will be condensed into Specialty 651.

- **Specialty 661: Non-Medical Transportation - BI/CIH/CMHS/EBD/Developmental Disabilities (DD)/SLS**
 - Specialty 660: Non-Medical Transportation - BI/CIH/CMHS/EBD will be condensed into Specialty 661.

Providers will be automatically enrolled into the updated specialties. Refer to the [Home and Community-Based Services Waivers web page](#) for more information regarding HCBS waivers.

Prohibition of Concurrent Delivery of Home Health Services and Home and Community-Based Services (HCBS) Developmental Disabilities Waiver

Effective April 29, 2026, the Department of Health Care Policy and Financing (the Department) implemented a claims system update to align with existing policy regarding Certified Nurse Assistant (CNA) and Individual Residential Services and Supports (IRSS) services.

Long-Term Home Health (LTHH) and IRSS providers are reminded that state regulations ([10 CCR 2505-10 8.520.5.B.13.g and 8.7541.C.4](#)) prevent the concurrent provision of LTHH CNA services and IRSS.

CNA services billed under the following revenue codes are not billable if the member has an active IRSS benefit on the date of service:

- 0570, 0571, 0572, 0579

IRSS claims will deny if billed on a date of service (DOS) where a CNA service has already been reimbursed.

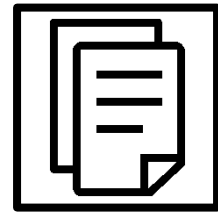
Providers are encouraged to carefully review service plans for members, work with case managers and coordinate accordingly to ensure CNA and IRSS services are not submitted for the same member on the same dates of service.

Contact the [Provider Services Call Center](#) with any general questions regarding billing or claims.

Contact HCPF_HCBS_Questions@state.co.us with any questions regarding policy.

Update for 2026 Base Wage and Workforce Reporting Compliance

Provider agencies must maintain current and accurate contact information in the [Provider Web Portal](#) to ensure receipt of all required communications related to Base Wage Reporting. This includes reporting instructions, required forms, deadlines and any follow-up requests. Failure to maintain up-to-date contact information may result in missed communications and potential non-compliance with reporting requirements.



Agencies are encouraged to review and update contact information in the Provider Web Portal no later than June 1, 2026, by following the step-by-step instructions in the [Provider Maintenance Guide](#).

Provider agencies are required to submit detailed information for each Direct Care Worker (DCW), including wage rates, hours worked, benefits, employment status and other wage-related data within the specified timeframes and using the required forms.

To stay informed about Base Wage Reporting and other updates, provider agencies are encouraged to:

- Subscribe to the [Office of Community Living Long-Term Services and Supports Newsletter](#)
- Join the mailing list for Monthly Notices of [Released Memos](#)
- Regularly review [Provider Bulletins](#)

Contact hcpf_basewage@state.co.us with questions, feedback or suggestions regarding the reporting process.

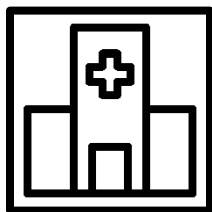
Hospital Providers

General Updates

Hospital Stakeholder Engagement Meeting

Bi-monthly Hospital Engagement meetings will be hosted by the Department to discuss current topics regarding ongoing rate reform efforts and operational concerns. [Sign up to receive the Hospital Stakeholder Engagement Meeting newsletters.](#)

- The next Hospital Stakeholder Engagement meeting is set for **Friday, July 10, 2026, from 1:00 p.m. to 3:00 p.m. Mountain Time** and will be hosted virtually.



Visit the [Hospital Stakeholder Engagement Meeting web page](#) for more details, meeting schedules and past meeting materials.

Contact Della Phan at Della.Phan@state.co.us with any questions or topics to be discussed at future meetings. Advanced notice will provide the Facility Rates Section time to bring additional Department personnel to the meetings to address different concerns.

Rural Health Clinic Stakeholder Engagement Meeting

A meeting for Rural Health Clinics (RHCs) has been scheduled for July 9, 2026, from 1:00 p.m. to 2:00 p.m. Mountain Time. Topics of discussion will include an overview of the Rural Health Clinic payment methodology for both hospital-based and freestanding RHCs and operational concerns impacting RHC billing or payment.

Visit the [Rural Hospital and Rural Health Clinic web page](#) for more details, meeting schedules and past meeting materials.

Contact Andrew Abalos at Andrew.Abalos@state.co.us with any questions or topics requested for discussion at this meeting.

Immunization Providers

Updates and Reminders

Health First Colorado covers vaccine counseling visits in which healthcare providers talk to families about the importance of vaccination. Health First Colorado also covers and will reimburse for stand-alone vaccine counseling visits. Providers should not bill for the vaccine counseling code and the vaccine administration code on the same date of service when vaccine administration codes are inclusive of counseling.

Effective immediately, providers should bill Current Procedural Terminology (CPT) codes 90482, 90483 or 90484 for visits in which healthcare providers give counseling about the importance of vaccination. CPT codes G0310, G0311, G0312, G0313, G0314 and G0315 will no longer be reimbursed for dates of service after June 30, 2026.



Rates can be found on the [Provider Rates & Fee Schedule](#) web page under the [Immunization Rate Schedule](#) section. Further vaccine billing guidance can be found in the [Immunizations Billing Manual](#).

Contact Christina Winship at Christina.Winship@state.co.us with any Vaccine Policy questions. Contact the [Provider Services Call Center](#) for assistance with claims and billing.

Managed Care Providers

New Managed Care Provider Escalation Request Form

Effective May 1, 2026, the Department will transition to a new platform for the [Managed Care Provider Escalation Request Form](#). The new link is available on the [Provider Contacts web page](#). Update records accordingly.

This form is for use by providers that are contracted with or attempting to contract with one of the following managed care entities (MCEs): Regional Accountable Entities (RAEs), Managed Care Organizations (MCOs), and/or Child Health Plan *Plus* (CHP+) MCOs.

It is recommended to contact the directly contracted MCE(s) first to resolve an issue or concern. It is understood that not all issues or concerns may be resolved this way. Complete the escalation request form for further support to resolve an issue or concern.

Outpatient Therapy

Plan of Care Signature Requirement Revisions Effective May 15, 2026

On March 13, 2026, the Medical Services Board voted to approve revisions to the Plan of Care (POC) Signature Requirements within the Medical Assistance Rule at [10 CCR 2505-10, Section 8.200](#). These revisions will become effective May 15, 2026. The updates align state policy with federal regulations at 42 C.F.R. § 424.24(c)(5) and will allow for greater flexibility in POC certification requirements. Under the revised rule, a physician's signature on the POC is no longer required when there is an existing physician order or referral and the therapist maintains documentation demonstrating that the POC was delivered to the physician's office within 30 days of completion of the initial evaluation.



This change applies to outpatient physical therapy, occupational therapy, speech language pathology and audiology services.

Contact Devinne Parsons at Devinne.Parsons@state.co.us with any questions related to Outpatient Physical, Occupational, Speech Therapy and Audiology services.

Pharmacy and All Medication Prescribers

LexisNexis Pharmacy Prescriber Validation

LexisNexis, which provides weekly prescriber validation files to the Pharmacy Benefit Management System (PBMS), recently informed the PBMS vendor, MedImpact, that due to a

recent privacy process update, a larger-than-expected volume of prescribers were intentionally excluded from the data files.

Because the LexisNexis prescriber database is used to validate submitted prescriber information in the PBMS, pharmacies may experience a higher-than-usual volume of claim denials related to prescriber data validation reject codes.

Prescribers who believe they were erroneously excluded from the LexisNexis prescriber database, or who wish to opt back in to data sharing, may send an email to privacydesk@lexisnexisrisk.com with the prescriber's National Provider Identifier (NPI), Drug Enforcement Agency (DEA) number and practice address. It may take up to ten days to process the request.

Contact the [Pharmacy Help Desk](#) for additional information or assistance with prescriber validation rejections and pharmacy claim processing.

Physician Services

Free Screening, Brief Intervention and Referral to Treatment (SBIRT) Training for Health First Colorado Providers

Free Screening, Brief Intervention and Referral to Treatment (SBIRT) training for Health First Colorado providers is provided through partnership with Peer Assistance Services (PAS), Inc. The SBIRT program promotes prevention and early intervention efforts through in-person, online and virtual training; technical assistance and hands-on SBIRT implementation.

Providers are required to participate in training that provides information about the implementation of evidence-based protocols for screening, brief interventions and referrals to treatment to directly deliver screening and intervention services.

Face-to-face training and consultations are available through various entities such as [SBIRT Colorado](#), [Carina Health Network](#) (formerly Colorado Community Managed Care Network) and the [Emergency Nurses Association](#).

Elevate SBIRT and motivational interviewing skills with Peer Assistance Services' new self-paced interactive practice scenarios. [Create a free account](#) to access a risk-free practice environment and engage in conversations with a patient about substance use. These simulations use guided prompts to walk through each interaction, improving the delivery and effectiveness of brief interventions.

Register for an upcoming SBIRT training at the [PAS training calendar](#). The shared goal is to promote SBIRT as a standard of care throughout Colorado. Refer to the [SBIRT Billing Manual](#) to learn more about best billing practices.

Contact Janelle Gonzalez at Janelle.Gonzalez@state.co.us with questions.

Lactation Support Services

The following enrolled provider types (PT) may provide and bill for Lactation Support Services (S9443) under their existing enrollment, provided it is within their scope of practice according to state licensing requirements and laws, and the provider has training in advanced lactation support:



- Certified Professional Midwife (CPM) / Direct Entry Midwife (DEM) - PT 69
- Certified Midwife (CM) - PT 80
- Certified Nurse Midwife (CNM) - PT 22
- Licensed Physician (MD) - PT 05
- Licensed Osteopathic Physician (DO) - PT 26
- Licensed Physician Assistant (PA) - PT 39
- Licensed Advanced Practice Nurse (APN) - PT 41
- Licensed Registered Nurse (RN) (enrolled as a Non-Physician Practitioner - Individual) - PT 24
- Certified Registered Nurse Anesthetist (CRNA) - PT 40
- Occupational Therapist (OT) - PT 28
- Speech-Language Pathologist (SLP) - PT 27

Certification Requirements

Providers must maintain a current lactation certification and submit documentation through the [Provider Web Portal](#).

Acceptable certifications include:

- International Board-Certified Lactation Consultant (IBCLC), issued by the International Board of Lactation Consultant Examiners (IBLCE)
- Certified Lactation Counselor (CLC), issued by the Academy of Lactation Policy and Practice (ALPP)
- Certified Lactation Educator (CLE), issued by the Childbirth and Postpartum Professional Association (CAPP)

Refer to the [Policy Guidance for Licensed Providers Rendering and Billing Lactation Support Services \(S9443\)](#) for step-by-step instructions on submitting certification documentation.

Refer to the [Lactation Support Services Billing Manual](#) for further information.
Contact hcpf_maternalchildhealth@state.co.us with any questions.

Primary Care Medical Providers

Access Stabilization Payment Update

The Department had planned to implement Access Stabilization Payments to support small, rural and pediatric primary care medical providers (PCMPs) who are historically unable to participate in value-based payments. This was in response to stakeholder feedback received during the design of Phase III of the Accountable Care Collaborative (ACC). ***At this time, Access Stabilization Payments have been suspended.***



New federal legislation, H.R.1 (One Big Beautiful Bill Act), significantly alters Medicaid financing, administrative requirements and eligibility processes, which increases state-level financial obligations. Colorado is also facing unprecedented state budget constraints and the Governor's office required reductions across state agencies. As a result, the Department proposed delaying the implementation of this new program as a cost containment and sustainability measure. Given the nature of the situation, difficult decisions must be made to balance the state budget, and the state legislature has voted to suspend this program for this reason.

The Department recognizes the importance of this program and the need to support providers serving these populations.

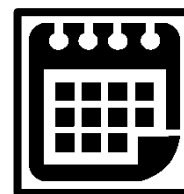
Eligible PCMPs should have received notification of the suspension of this program from their Regional Accountable Entity (RAE). PCMPs with additional questions can contact their RAE or the Department at HCPF_primarycarepaymentreform@state.co.us.

Transportation Providers

Additional Important Reminders for TransDev Providers

Timely closeout of completed trips is necessary for TransDev to process claims and issue payments. NEMT providers are required to close all trips within 30 days of trip completion.

Beginning on May 1, 2026, Transdev is requiring all NEMT providers to use either a tablet or smartphone with the ECOLane software installed for trip acceptance and closeout. All trips completed on or before June 30, 2026, must be closed out with Transdev by July 17, 2026. Any trips closed after July 17 will see a delay in payment issuance from either the Department or MediDrive, LLC.



Contact NEMT@state.co.us with any questions.

MediDrive Enrollment for TransDev Network Providers Only

Existing TransDev network NEMT providers must begin the enrollment process with MediDrive immediately to ensure uninterrupted service across the current nine (9) metro area county broker service area.

How to Begin - Start the application at [MediDrive Enrollment](#).

What Needs to Be Done - To streamline the transition and reduce administrative burden, providers currently active in the TransDev network do **not** need to upload their driver and vehicle rosters when submitting the initial contracting application. Simply complete the application with company information.

The Department will transfer existing driver and vehicle rosters directly from TransDev to MediDrive on behalf of the providers.

- **Critical:** Providers must contract with MediDrive using the exact same business name and NPI/Medicaid ID that was used with TransDev. This ensures records transfer accurately.

Keeping Records Current - All driver and vehicle rosters must remain current and complete in the TransDev ProCredEx system throughout this transition. This includes all required:

- Licensing
- Insurance
- Vehicle inspections
- Driver credentials and certifications
- Any other compliance documentation

Routine roster maintenance and credentialing with MediDrive will begin after July 1, 2026.

Providers need to review their files carefully before submitting enrollment materials to avoid delays. Additional information will be provided as it becomes available.

For the NEMT providers operating **outside** the broker service area, do **not** enroll with or seek credentialing from MediDrive at this time. Transition resources are being focused on the existing nine (9) metro county network.

Additional information will be provided as it becomes available. Contact NEMT@state.co.us with any questions regarding the new application process with MediDrive.

Non-Emergent Medical Transportation (NEMT) Providers Upcoming Broker Communications

All actively enrolled Non-Emergent Medical Transportation (NEMT) providers should expect communications from the Department in the coming weeks regarding new broker timelines, key transition information and enrollment details.



Operational memo OM26-018, outlining the broker transition plan, has been published and made available on the [Operational Memo web page](#).

Information will be shared by the Department through email notifications, newsletters and the monthly Provider Bulletin as the transition begins and continues.

Non-Emergent Medical Transportation (NEMT) Statewide Broker Transition Stakeholder Webinar

Stakeholders are invited to [register](#) for a webinar on **Thursday, May 14, 2026, from 10:00 a.m. MT to 12:00 p.m. MT** to review the Non-Emergent Medical Transportation (NEMT) statewide broker transition.

An [operational memo](#) was published on March 27, 2026, outlining the transition from TransDev to MediDrive, including key implementation dates and requirements. This webinar will review that information, highlight what stakeholders need to know ahead of the Phase 1 implementation and offer an opportunity for questions and clarification. MediDrive will also demonstrate its scheduling systems during the webinar.

NEMT is a benefit provided to Health First Colorado members who do not have any other mode of transportation to medical appointments.

Meeting Details

- **Meeting date and time:** Thursday, May 14, 2026, from 10:00 a.m. to 12:00 p.m. MT.
- **Intended audiences:** NEMT transportation providers, counties, facilities, member advocates, tribes and other impacted stakeholders.
- **Registration and location:** The meeting will be virtual via Zoom. [Register](#) in advance or at the start of the webinar. Registrants will receive a unique link to join the meeting. Links are tied to individual registration and will not work for anyone else. A recording of the webinar will be posted to the [Non-Emergent Medical Transportation \(NEMT\) web page](#).
- **Meeting Materials:** Stakeholders are encouraged to review the [NEMT operational memo](#) ahead of time.

Meeting Accommodation and Language Access Notice

Auxiliary aids and services for individuals with disabilities and language services for individuals whose first language is not English may be provided upon request. Notify the meeting organizer Ryan Lazo at hcpf_stakeholders@state.co.us or the Civil Rights Officer at hcpf504ada@state.co.us, at least one (1) week before the meeting to make arrangements.



Aviso sobre adaptaciones para la reunión y acceso lingüístico

Las ayudas y servicios auxiliares para individuos con discapacidades y servicios de idiomas para individuos cuyo idioma materno no sea inglés pueden estar disponibles por solicitud. Comuníquese con organizador de reuniones Ryan Lazo a hcpf_stakeholders@state.co.us o con el oficial de derechos civiles a hcpf504ada@state.co.us, al menos una (1) semana antes de la reunión para hacer los arreglos necesarios.

Reminder to All Non-Emergent Medical Transportation (NEMT) Providers

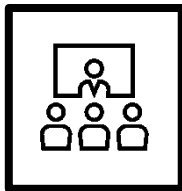
Enrolled Non-Emergent Medical Transportation (NEMT) providers are reminded that they must follow these policies:

- Providers must not directly solicit individual members known to have already established NEMT service with another provider (10 CCR 2505-10 8.014.3.B.6.c.).
- Providers must only provide NEMT services appropriate to their current licensure(s) and within the geographic limitations applicable to that licensure (10 CCR 2505-10 8.014.3.B.6.d.).
- Providers must ensure that all vehicles and auxiliary equipment used to transport members meet all applicable federal, state and local safety inspection and maintenance requirements (10 C.C.R. 2505-10 8.014.3.B.6.f.).
- NEMT documents must **not** be faxed, emailed or mailed to the Department. These forms are to be exchanged only between the NEMT provider and the referring or treating provider, as applicable.
 - Documents sent to the Department which are not part of the claim submission or approval process detailed in the [NEMT billing manual](#) will not be forwarded to the appropriate provider. Submitting forms to the Department may result in delays and administrative burden.
 - Ensure all NEMT documentation is sent through the correct provider-to-provider process.
- Providers are reminded to contact the [Provider Services Call Center](#) for billing and general assistance, and not the main phone number for the Department.

Contact NEMT@state.co.us with any questions.

Provider Training Sessions

May 2026 Schedule



Providers are invited to sign up for provider training sessions. All sessions are held via webinar on Zoom, and registration links are shown in the calendar below. *The availability of training sessions varies monthly.* Descriptions of available training sessions, calendar registration links and training-specific slide decks are available on the [Provider Training web page](#).

The following training sessions focused on Health First Colorado will be offered in May:

- **Beginning Billing Training**

Beginner billing training provides a high-level overview of member eligibility, claim submission, prior authorizations, Department website navigation, [Provider Web Portal](#) use and more. This training is appropriate for providers who submit either professional claims (CMS 1500) or institutional claims (UB-04), but **this training is not appropriate for Home and Community-Based Services (HCBS) providers**. The Department will be offering a beginner billing training specific to HCBS providers in June 2026.

- Audience: Staff who submit claims, are new to billing Health First Colorado services or who need a billing refresher course should consider attending one of the beginner billing training sessions.
- Time: One and a half (1.5) hour presentation / half (0.5) hour Q&A

- **Provider-Specific Billing Training - Indian Health Services (IHS)**

Provider-specific training delivers an overview of benefits, covered services, and provider eligibility unique to each provider type.

- Audience: This training applies to specific provider types.
- Time: One (1) hour presentation / half (0.5) hour Q&A

- **Billing Training: Medicare and Third-Party Liability**

This focused micro-training addresses billing Medicare and third-party liability (TPL) (e.g., commercial and private insurance) as primary payers, including detailed information on Medicare lower-of pricing logic and timely filing guidelines.

- Audience: This training is not relevant to Home and Community-Based Services (HCBS) and Non-Emergent Medical Transportation (NEMT) providers. This training applies to all other provider types and is recommended after attending beginner billing training.
- Time: One (1) hour presentation / half (0.5) hour Q&A



Live Webinar Registration

Click the title of the desired provider training session in the calendar to register for a webinar. An automated response will confirm the reservation. Webinars may end early. Time has been allotted for questions at the end of each session.

May 2026 Training Calendar

Monday	Tuesday	Wednesday	Thursday	Friday
				1
4	5 Beginner Billing Training 10:00 a.m. - 12:00 p.m. MT	6	7 Billing Medicare & Third-Party Liability 10:00 a.m. - 11:30 a.m. MT	8
11 Provider-Specific Training: Indian Health Services 1:00 p.m. - 2:30 p.m. MT	12	13	14	15
18	19	20	21	22
25	26	27	28	29

Note: All training sessions offer guidance for Health First Colorado only. Providers are encouraged to contact the Regional Accountable Entities (RAEs), Child Health Plan *Plus* (CHP+) and Medicare for enrollment and billing training specific to those organizations. Training for the Care and Case Management (CCM) system will not be covered in these training sessions. Visit the [CCM System web page](#) for CCM-specific training and resources.

Refer to the Provider Web Portal Quick Guides located on the [Quick Guides web page](#) for more training materials on navigating the Provider Web Portal.

Upcoming Holidays

Holiday	Closures
Memorial Day May 25, 2026	State Offices, AssureCare, Acentra, DentaQuest, Gainwell and Optum will be closed. MedImpact will be open. Capitation cycles for managed care entities may potentially be delayed. The receipt of warrants and EFTs may potentially be delayed due to the processing at the United States Postal Service or providers' individual banks.

Provider Services Call Center

1-833-468-0362

